CHED POLICY BRIEF

NMAT Cut-Off Score Policy: Evidence-Based Analysis for the CHED CMO Amendment

Review of the proposed amendment to Section 17.5 of CHED Memorandum Order No. 18, series of 2016

PREPARED FOR	CHED Commissioners, Policy Officers, and Institutional Stakeholders
EVIDENCE BASE	NMATExodus.parquet — 178,927 NMAT examinee records, 2006–2018
PURPOSE	Descriptive evidence to inform review of proposed NMAT cut-off provisions

Scope note: This policy brief reports descriptive evidence only. It does not make regulatory recommendations, assign compliance labels, or determine institutional eligibility.

1. Executive Summary

The evidence base covers 178,927 National Medical Admission Test (NMAT) records from 2006 to 2018 and indicates that most best-record examinees already meet the percentile thresholds addressed by the proposed amendment. Nationally, 69.9 percent of best-record examinees scored at or above the 30th percentile (B4+), while 60.4 percent scored at or above the 40th percentile (B5+); 9.5 percent fell in the B4-only band, meaning they met the 30th-percentile floor but not the 40th-percentile standard. Among Public-institution examinees, 64.9 percent already met B5+, compared with 59.2 percent of Private-institution examinees. In the observable cohort, the NMAT-to-PLE-passer linkage rate increased from 22.9 percent in B4 to 46.3 percent in B5, although this measure is not an official PLE passing rate. The available evidence is descriptive: it does not identify GIDA/IP status, actual medical-school enrollment, complete PLE outcomes, or institution-level PHEI eligibility under the proposed policy.

2. Data Foundation

This analysis uses `NMATExodus.parquet`, the final product of a four-stage pipeline covering data cleaning, PLE matching, statistical analysis, and citizenship integration. The file contains 178,927 NMAT examination records across 54 columns for examination years 2006 through 2018. The analysis uses the dashboard's best-record approach, which selects one NMAT record per person to prevent repeat attempts from inflating person-level counts.

Score and cohort definitions

- **TRUE raw scores.** The NMAT consists of two parts and eight subtests. The originally stored total in the source CEM data was inconsistent with the sum of the eight component scores in 42.2 percent of CEM records (107,422 of 254,308 records). The pipeline therefore recalculated `TotalRawScoreTRUE` from the eight components; the independently calculated source total had zero mismatches against the recalculated total.
- **Percentile bins.** Examinees are classified into ten score bins, B1 through B10. B4 denotes the 30th–39th percentile band, while B5 denotes the 40th–49th percentile band. “B4+” means at or above the 30th percentile; “B5+” means at or above the 40th percentile.
- **Deterministic PLE matching.** PLE-passers were linked through exact normalized-name or application-number matches; no fuzzy matching was used. Multiple exact-name candidates were resolved using a minimum NMAT-to-PLE year gap and additional selection rules. Application-number matches were traceable to a specific NMAT record and underwent a supplementary name-consistency review.
- **Observable cohort.** PLE-linked analysis is restricted to best-record records with NMAT year 2014 or earlier (n = 64,501). This restriction gives examinees time to complete medical school and take the PLE, reducing right-censoring bias for more recent NMAT cohorts.
- **Repeat-taker control.** The dashboard’s best-record analytic subset contains 133,804 flagged NMAT records, representing 133,558 distinct `PERSONKEY` values. Of these unique examinees, 33,713 (25.0 percent) took the NMAT more than once, with up to nine attempts. The dashboard uses the best-record subset for threshold and institution-level counts; for confirmed PLE passers, it selects the linked NMAT attempt and, for others, the highest-percentile attempt with the most recent year as a tiebreaker.

3. Key Findings for Policy

3.1 Threshold volumes

Among 130,818 best-record records with a valid percentile bin, 91,409 (69.9 percent) scored at or above B4, and 78,944 (60.4 percent) scored at or above B5. The difference is 12,465 B4-only examinees, or 9.5 percent of the best-record population. This B4-only group is the record-based analytic population directly affected when a policy distinguishes between a 30th-percentile floor and a 40th-percentile standard.

Threshold	Examinees	Share of best-record examinees with valid percentile bins
B4+ — at or above the 30th percentile	91,409	69.9%
B5+ — at or above the 40th percentile	78,944	60.4%
B4-only — 30th–39th percentile	12,465	9.5%

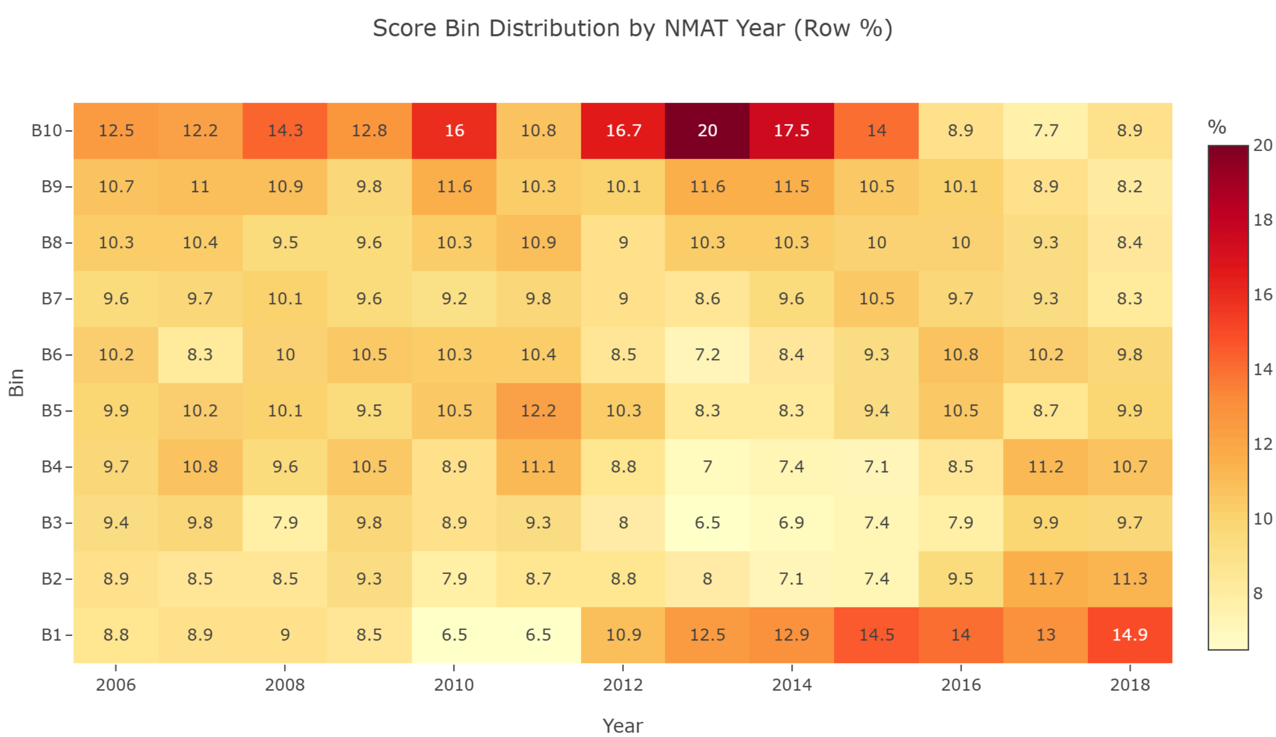

Figure 1. Score-bin distribution by NMAT year

3.2 PLE-passer linkage gradient

Within the observable cohort, NMAT-to-PLE-passer linkage rises across score bins, from 8.3 percent in B1 to 76.1 percent in B10. A linkage rate is the proportion of NMAT examinees later matched to a PLE passer record. It is **not** an official PLE passing rate because the dataset does not contain all PLE takers or PLE failures.

The policy-relevant change occurs between B4 and B5. The linkage rate is 22.9 percent for examinees in B4 (30th–39th percentile) and 46.3 percent for examinees in B5 (40th–49th percentile), an increase of 23.4 percentage points.

Score bin	Percentile range	Observable cohort	Confirmed PLE passers	NMAT-to-PLE-passer linkage rate
B1	0–9	6,104	505	8.3%
B2	10–19	5,254	830	15.8%
B3	20–29	5,228	997	19.1%
B4	30–39	5,741	1,312	22.9%
B5	40–49	6,229	2,882	46.3%
B6	50–59	5,831	2,992	51.3%
B7	60–69	5,942	3,359	56.5%
B8	70–79	6,355	3,819	60.1%
B9	80–89	6,854	4,595	67.0%
B10	90–100	9,657	7,352	76.1%

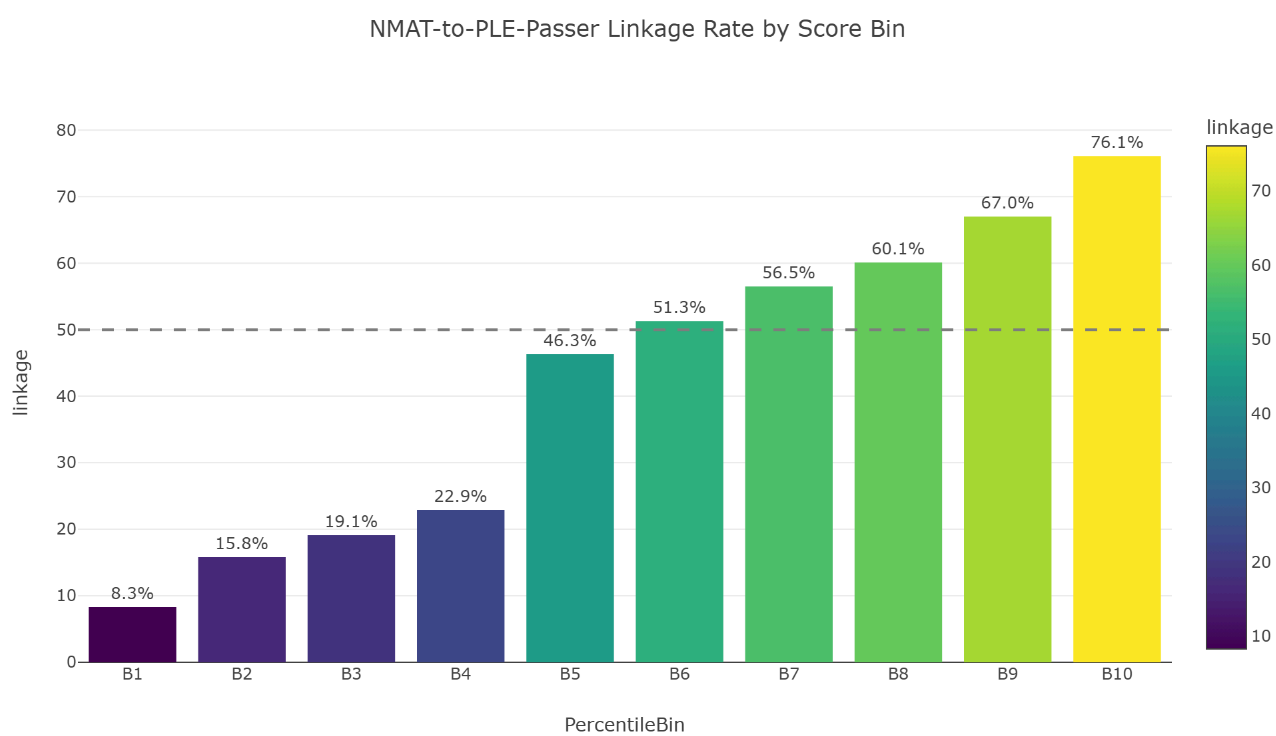

Figure 2. NMAT-to-PLE-passer linkage rate by percentile band

3.3 Institutional patterns

Public-institution examinees meet B5+ at a somewhat higher rate than Private-institution examinees. Of 26,937 best-record Public-institution examinees, 17,482 (64.9 percent) scored at or above B5, compared with 59,546 of 100,584 Private-institution examinees (59.2 percent). The B4-only group represents 8.3 percent of Public examinees and 9.9 percent of Private examinees.

Undergraduate institution type	Best-record examinees	B4+	B5+	B4-only
Public	26,937	19,709 (73.2%)	17,482 (64.9%)	2,227 (8.3%)
Private	100,584	69,504 (69.1%)	59,546 (59.2%)	9,958 (9.9%)
Foreign	1,862	1,285 (69.0%)	1,109 (59.6%)	176 (9.5%)
Not Specified	1,352	911 (67.4%)	807 (59.7%)	104 (7.7%)

In the observable cohort, NMAT-to-PLE-passer linkage is 50.1 percent for Public-institution examinees, 44.7 percent for Private-institution examinees, and 22.1 percent for Foreign-institution examinees. The data does not identify the reasons for these differences.

UNITYPE identifies the examinee's recorded **undergraduate institution type**. It does not identify the medical school to which the examinee applied, was admitted, or enrolled. The figures in this section should therefore not be read as direct measures of SUC or PHEI medical-school performance.

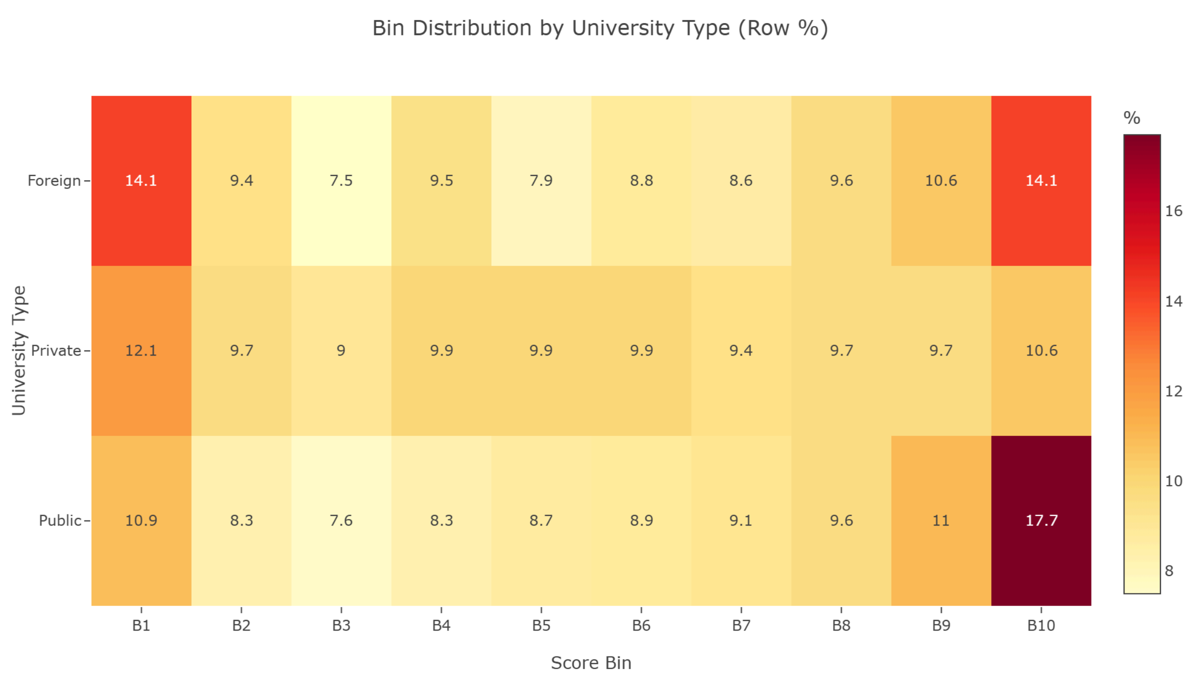

Figure 3. NMAT score-bin distribution by undergraduate institution type

3.4 Data quality: clean-subset stress test

The dashboard tests whether the main results are sensitive to match quality by applying the strictest available criteria: best-record status, deterministic PLE linkage, an NMAT-to-PLE gap of at least five years, and Filipino nationality. This clean subset contains 27,151 records across all score bins; 23,357 of these records are in B5 or above, and the median NMAT-to-PLE year gap for the B5+ subset is six years.

The clean B5+ subset is composed of 73.6 percent Private-institution examinees, 24.5 percent Public-institution examinees, 0.7 percent Foreign-institution examinees, and 1.2 percent Not Specified. These results support the dashboard's conclusion that the principal B5 and institutional patterns are not driven solely by less restrictive PLE-matching criteria. They do not establish an official PLE pass rate or institution-level performance measure.

A supplementary record-level audit found that 15 of 29,258 deduplicated confirmed PLE-passer best-records (0.1 percent) had a data-quality flag, principally related to duplicate best-record assignment. Excluding these records did not change the B4-to-B5 gradient in the audit population, which remained 4.5 percentage points.

Clean subset (all bins)	B5+ in clean subset	Share of observable cohort	Median PLE year gap
27,151	23,357	36.2%	6 yrs

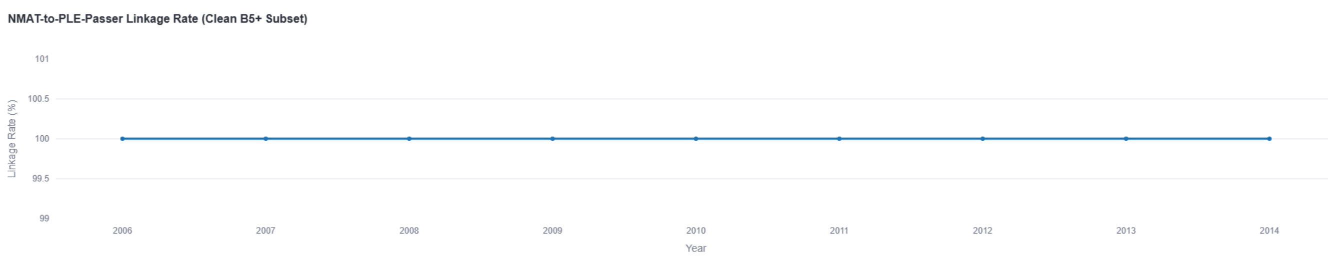

Figure 4. Yearly NMAT-to-PLE-passer linkage rate in the clean B5 subset

3.5 Foreign examinee context

The dataset contains two different measures that may be described as "foreign," and they should not be combined. **Citizenship-based counts** identify 32,501 verified foreign NMAT records (18.2 percent of 178,927 records) across 90 nationalities, while 146,413 records are classified as Filipino. **Foreign `UNITYPE`** identifies 1,894 best-record examinees who attended an undergraduate institution classified as foreign; this is a separate measure based on undergraduate institution, not citizenship.

India accounts for 26,490 verified foreign records (85.1 percent), followed by Nepal (1,158; 3.7 percent), Thailand (1,062; 3.4 percent), the United States (839; 2.7 percent), and Nigeria (639; 2.1 percent). These are NMAT examinee records, not medical-school enrollment records.

Measure	Count	Share / interpretation
Verified foreign nationals, all NMAT records	32,501	18.2% of 178,927 records; classified by citizenship
Filipino nationals, all NMAT records	**146,413**	81.8% of 178,927 records; classified by citizenship
Foreign undergraduate institution type, best-record examinees	1,894	1.4% of 133,804 best-record records; classified by undergraduate institution

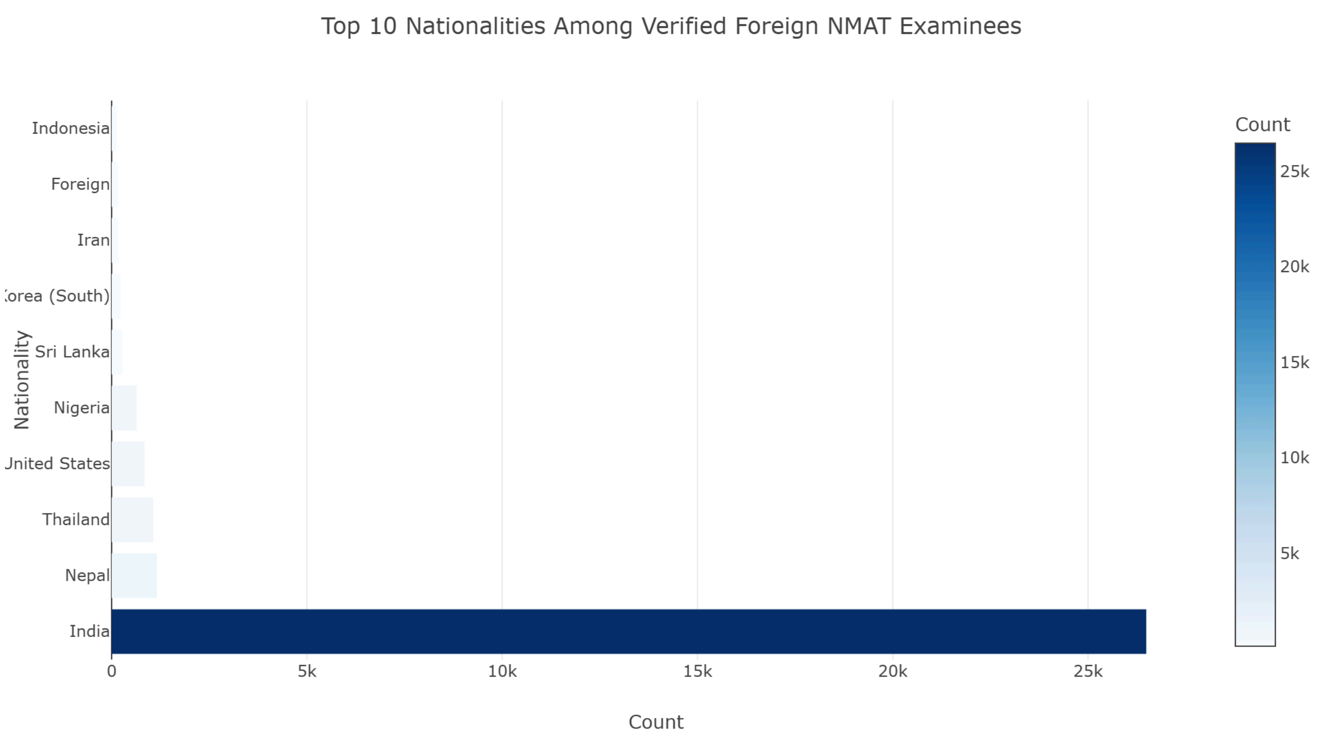

Figure 5. Top 10 nationalities among verified foreign NMAT examinees

4. Policy Implications

The observations below describe what the available evidence shows in relation to the proposed amendment. They are not regulatory recommendations, compliance determinations, or institutional eligibility assessments.

4.1 Forty-percentile SUC standard

The draft amendment maintains the 40th-percentile NMAT requirement for Filipino applicants to SUCs, while permitting SUC Governing Boards to set higher thresholds. Historically, 17,482 of 26,937 Public-institution examinees (64.9 percent) met B5+. On the available data, the majority of this examinee group already meets the proposed standard.

4.2 Thirty-percentile GIDA/IP exception

The draft amendment permits an SUC to admit applicants in the B4-only range if they belong to a GIDA or IP group and provide the required documentation. The dataset identifies 2,227 B4-only Public-institution examinees, representing 8.3 percent of Public best-record examinees. However, the dataset has no GIDA-residence or IP-membership variable; it cannot determine how many B4-only examinees could satisfy the proposed documentation requirement.

4.3 PHEI threshold differentiation

The draft amendment allows a PHEI to set an NMAT floor of at least the 30th percentile only if its five-year PLE performance is above the national average; PHEIs below that benchmark would have a 40th-percentile minimum. The dashboard's examinee-level evidence shows a substantial linkage increase from B4 (22.9 percent) to B5 (46.3 percent), and the clean-subset analysis supports the robustness of the principal B5 pattern. The dashboard does not calculate PHEI-level PLE passing rates, compare institutions with a national five-year benchmark, or assign eligibility or compliance labels; it therefore cannot determine which PHEIs meet the proposed condition.

4.4 Foreign enrollment cap and composite ranking

The draft amendment caps foreign-student enrollment at SUCs at 10 slots per incoming freshman class beginning AY 2026–2027 and provides for composite or weighted ranking of foreign applicants. The dataset describes the NMAT examinee pool by citizenship and score but does not record admission decisions, actual enrollment, GWA, interview results, or other non-NMAT criteria. It therefore cannot assess compliance with the 10-slot cap or estimate how a composite ranking system would change admission outcomes.

5. Limitations

- **NMAT-to-PLE-passer linkage is not a PLE pass rate.** The dataset identifies examinees who were later matched to PLE passer records but does not identify all PLE takers or failures. Official PLE passing rates require separate PRC outcome data.
- **GIDA/IP status is unavailable.** No data field identifies GIDA residence or Indigenous Peoples membership. The evidence cannot estimate the number of applicants who would be eligible for the proposed SUC B4 exception.
- **The data records NMAT examinees, not medical-school enrollment.** Examinees meeting a threshold represent a potential applicant pool, not actual admission, enrollment, completion, or graduation figures.
- **Institutional admission and capacity factors are absent.** The dataset does not include GWA, interview scores, medical-school capacity, or other admission criteria. It cannot evaluate a composite ranking system.
- **Foreign examinee measures do not assess the enrollment cap.** Citizenship and undergraduate-institution classifications are not enrollment counts and cannot be used to assess adherence to the SUC 10-slot limit.

- **No institution-level compliance labels are calculated.** The dashboard does not determine PHEI eligibility for a 30th-percentile floor, compliance with an NMAT cut-off, or institution-level risk.
- **Best-record counts are record-based.** The dashboard uses 133,804 `ISBESTNMATRECORD`-flagged records for threshold and institution-level analysis, representing 133,558 distinct `PERSONKEY` values. Reported threshold volumes therefore use the dashboard's record-based analytic denominator; distinct-person counts are stated separately.

6. Conclusion

The historical NMAT evidence indicates that most best-record examinees meet the thresholds addressed by the proposed amendment and that Public-institution examinees meet the 40th-percentile threshold at a comparatively high rate. It also shows a pronounced association between the B4 and B5 score bands and later PLE-passer linkage within the observable cohort, while the clean-subset analysis supports the robustness of the principal B5 pattern under stricter matching conditions.

The evidence is necessarily limited: it cannot identify GIDA/IP applicants, actual medical-school enrollees, complete PLE outcomes, or institutional PLE performance relative to the national benchmark. The findings should therefore be used as descriptive evidence on historical NMAT examinee performance alongside the additional administrative and outcome data required for implementation and monitoring of the proposed amendment.